INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Vemurafenib alone — advanced melanoma

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00102
Version 6

Reference treatment cycle

Treatment stages

01 **Ongoing oral treatment**
28-day reference cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	6
03	Symptoms and supportive care	11
04	Sources and further information	18

My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact the oncology team promptly for fainting or palpitations, eye pain or blurred vision, a blistering or rapidly spreading rash, severe sunburn, persistent vomiting or diarrhoea, reduced urine or yellow skin or eyes. Report new or changing skin growths even when the melanoma is responding.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

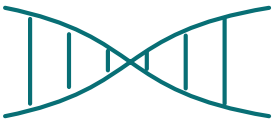
What each medicine does

To control advanced BRAF V600-mutated melanoma with a daily oral targeted medicine; benefit and tolerability are reviewed for the individual.

BRAF TARGETED TREATMENT

Vemurafenib

Zelboraf



Blocks an altered BRAF signal

Vemurafenib blocks the altered BRAF V600 protein that can drive melanoma-cell growth. A confirmed tumour BRAF V600 result is needed. Healthy tissues can also be affected; tablets need the same careful monitoring as treatment given in hospital.

Blocks an altered BRAF signal → Treatment plan and safety checks → Report new symptoms early

What to watch for

Report new skin growths, sunburn-like reactions, eye pain or blurred vision, palpitations or fainting.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding your diagnosis and treatment goal

Use your own pathology, stage and agreed plan alongside this guide.

Your diagnosis and stage

Record where the melanoma began, its pathology result and the sites involved now. This pathway treats melanoma that cannot be fully removed or has spread. Treatment after complete removal has a different aim and a separate plan.

The tumour test

The laboratory checks a sample of the melanoma for a BRAF V600 change. The result can identify a target for certain medicines. A change found in the tumour does not, by itself, establish an inherited condition.



Scans and local treatments

Add planned scans, any operation and any radiotherapy to the same calendar. Ask what each part is intended to achieve and which team arranges it.

New changes still matter

Tell your team about a changing mole, a persistent sore, a new lump or an unexplained symptom. Improvement in a known tumour does not explain a new problem elsewhere.

Your medicines and overall plan

The schedule

Vemurafenib twice daily, about 12 hours apart, every day. A 28-day review cycle does not mean a planned tablet-free week.

Important boundaries

This is vemurafenib monotherapy. It is not the cobimetinib combination and has no cobimetinib seven-day break. The team must plan any radiotherapy because BRAF treatment can increase radiation-related reactions.

Evidence and choices

At the 2011 analysis, six-month overall survival was 84% versus 64%; response was 48% versus 5%. These are group results from that study, not a prediction for you. Early historical analysis; later crossover and subsequent treatment affect longer-term comparisons. Dacarbazine comparator, not a modern BRAF/MEK combination or checkpoint regimen.

Decisions, support and useful records

Your priorities belong in the treatment discussion.

Benefits and alternatives

Ask what the treatment aims to achieve, what alternatives are suitable, how likely important benefits are in your situation and how treatment burden or uncertainty affects that choice. A trial average cannot promise an individual outcome.

Support alongside treatment

Supportive care addresses symptoms and daily life during cancer treatment. Palliative care can be part of that support; accepting it does not mean you must stop anticancer treatment.

A person you trust

Choose whether to involve a relative or supporter and what may be shared with them. Ask for an interpreter or a different information format. Check that you can explain the next dose, tests and urgent-contact plan in your own words.

Your document set

Keep the current treatment summary, medicine list, test plan and contact card together. Use the printed instructions when you need them; the QR code is an optional link to further information.

What the treatment evidence can and cannot tell you

Evidence needs to match the medicine, dose combination, disease stage and previous treatment.

CHAPTER 02

Your schedule, tests and appointments

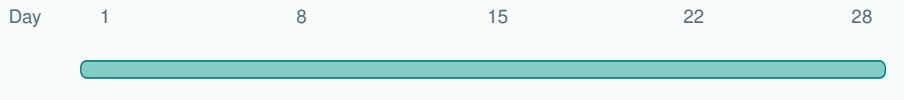
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment phases

Vemurafenib alone — advanced melanoma

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

28-DAY REFERENCE CYCLE

Ongoing oral treatment

Ongoing oral treatment **Repeat only as prescribed**

Vemurafenib twice daily, about 12 hours apart, every day. A 28-day review cycle does not mean a planned tablet-free week.

BASELINE → TREATMENT → FOLLOW-UP

Your testing and review plan

A reference cycle is not a personalised calendar. Tests listed as symptom-triggered or individual are not presented as mandatory tests at every visit. No universal imaging or tumour-marker interval is invented.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00102 v6: current regimen source · EMA Zelboraf product information

THE WHOLE TREATMENT JOURNEY

Vemurafenib alone — advanced melanoma: treatment and checks

Advanced melanoma

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms NCCP 00102	Confirm BRAF V600, blood count, kidney/ liver tests, medicine interactions and skin assessment. Current Zelboraf product information calls for a baseline electrocardiogram (ECG) and electrolytes including magnesium for all patients; this supplements the NCCP risk-based ECG row. Staging scans establish the sites of melanoma.	Blood count, kidney and liver tests before each cycle. NCCP specifies ECG every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Its skin-review row states week eight then every 12 weeks; current Zelboraf product information instead requires monthly prescriber skin examinations. The team should supply a reconciled written skin-check plan using the current product information.	Contact the oncology team promptly for fainting or palpitations, eye pain or blurred vision, a blistering or rapidly spreading rash, severe sunburn, persistent vomiting or diarrhoea, reduced urine or yellow skin or eyes. Report new or changing skin growths even when the melanoma is responding.	Current Zelboraf product information calls for monthly skin checks during treatment and for up to six months after stopping, or until another anticancer therapy starts. Head-and-neck checks every three months and chest computed tomography (CT) every six months during treatment screen for other cancers; these are distinct from the individual melanoma-response scan plan. Anal/ pelvic checks are considered at baseline/ end and when indicated. Record the team's coordinated surveillance plan rather than duplicating scans.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results NCCP 00102	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Written plan and contacts NCCP 00102	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

What each check is for

1 Ongoing oral treatment

28-day reference cycle

To control advanced BRAF V600-mutated melanoma with a daily oral targeted medicine; benefit and tolerability are reviewed for the individual.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

The usual starting dose is 960 mg twice daily, about 12 hours apart; the prescribed dose may differ after review. Swallow tablets whole with water. Vemurafenib can be taken with or without food, but do not regularly take both daily doses on an empty stomach. Keep a consistent routine and follow the pharmacy label.

STEP 2

Missed, vomited or held treatment

Take a missed dose only if at least four hours remain before the next dose. Otherwise skip it. Do not double a dose. If you vomit after taking it, do not take an extra dose; take the next dose at the usual time. Contact the team if vomiting continues or treatment has been held.

STEP 3

Appointments and supply

Before each visit, check four things: the blood-test booking, how you will get there, your medicine supply and any help you need. Ask who will review the results and tell you whether the next dose can go ahead.

STEP 4

Paper copies and contacts

Keep the handbook, short guide and treatment card in one place. Ask your team to fill in the real daytime and emergency numbers and print a replacement when the plan changes. A phone is optional.

Taking or receiving treatment safely

Keep the written prescription and actual treatment record together.

Other medicines

Give the team a complete list of prescribed, over-the-counter and herbal medicines, supplements and allergies. Ask before adding a new medicine; treatment interactions and symptom-masking can matter.

Practical arrangements

Write down where blood samples are taken, who receives the results and how you obtain the next appointment or medicine supply. Request a printed plan if that is easier to use.

After a treatment hold, change or completion

The next step should be written down, not assumed.

Follow-up and late effects

Current Zelboraf product information calls for monthly skin checks during treatment and for up to six months after stopping, or until another anticancer therapy starts. Head-and-neck checks every three months and chest CT every six months during treatment screen for other cancers; these are distinct from the individual melanoma-response scan plan. Anal/pelvic checks are considered at baseline/end and when indicated. Record the team's coordinated surveillance plan rather than duplicating scans.

A treatment summary

Ask for a treatment summary showing what you received, important side effects, admissions, the last dose and the follow-up team. Share the current version with your GP and other clinicians.

Your next review

Before leaving, confirm the next test and review, how you will receive results and whom to call sooner. Keep those numbers available after treatment ends.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Daily tablets / capsules	 **Days 1–28** Vemurafenib twice daily, about 12 hours apart, every day. A 28-day review cycle does not mean a planned tablet-free week.
Safety and symptom review	 **Days 1–28** Blood count, kidney and liver tests before each cycle. NCCP specifies ECG every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Its skin-review row states week eight then every 12 weeks; current Zelboraf product information instead requires monthly prescriber skin examinations. The team should supply a reconciled written skin-check plan using the current product information.

Schedule and review: Vemurafenib twice daily, about 12 hours apart, every day. A 28-day review cycle does not mean a planned tablet-free week.

Holds, missed treatment and phase changes: Take a missed dose only if at least four hours remain before the next dose. Otherwise skip it. Do not double a dose. If you vomit after taking it, do not take an extra dose; take the next dose at the usual time. Contact the team if vomiting continues or treatment has been held.

Why these bands stay broad: This is a reference schedule. Your prescribed dose, blood results, symptoms and the team's written plan determine whether treatment goes ahead. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00102 v6: current regimen source ↗ · EMA Zelboraf product information ↗

There are no guaranteed "safe days". The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Between-treatment notes

Day 1–28: At home each day

The usual starting dose is 960 mg twice daily, about 12 hours apart; the prescribed dose may differ after review. Swallow tablets whole with water. Vemurafenib can be taken with or without food, but do not regularly take both daily doses on an empty stomach. Keep a consistent routine and follow the pharmacy label. Take a missed dose only if at least four hours remain before the next dose. Otherwise skip it. Do not double a dose. If you vomit after taking it, do not take an extra dose; take the next dose at the usual time. Contact the team if vomiting continues or treatment has been held.

Support around the medicines

1 Supportive medicines and symptom plan

This is a minimal-emetic-risk oral regimen in the NCCP plan. Anti-sickness medicine is chosen to match symptoms and interactions; do not add over-the-counter medicines without checking. Agree a hydration and diarrhoea plan, and report inability to drink or persistent symptoms.

2 Skin and daily care

Use broad-spectrum UVA/UVB sun protection, protective clothing and lip protection; the product information specifies sun protection factor (SPF) 30 or higher. Sun sensitivity can be marked. Check skin regularly for new growths or changes, and keep scheduled dermatology reviews.

3 Fertility, pregnancy and breastfeeding

Use effective contraception during treatment and for at least six months after the final dose. Vemurafenib may reduce the effectiveness of hormonal contraception, so ask for an appropriate alternative or additional method. Discuss fertility and breastfeeding before treatment; tell the team immediately if pregnancy is possible.

Tests, results and treatment decisions

Safety tests and scans answer different questions.

Before treatment

Confirm BRAF V600, blood count, kidney/liver tests, medicine interactions and skin assessment. Current Zelboraf product information calls for a baseline ECG and electrolytes including magnesium for all patients; this supplements the NCCP risk-based ECG row. Staging scans establish the sites of melanoma.

During treatment

Blood count, kidney and liver tests before each cycle. NCCP specifies ECG every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Its skin-review row states week eight then every 12 weeks; current Zelboraf product information instead requires monthly prescriber skin examinations. The team should supply a reconciled written skin-check plan using the current product information.

What a result means

Ask which result is abnormal, whether the finding needs a repeat test and whether treatment is held, changed or stopped. Do not use a reference range or one normal result to decide independently that a dose is safe.

What to expect between visits

A quiet treatment day does not rule out later problems.

Supportive medicines

A useful daily record

Record symptoms, temperature when unwell, bowel changes, fluid intake when reduced, missed doses and any medicines used for relief. Note how symptoms affect eating, walking, sleep and normal activities; this helps the team judge severity.

When life is affected

Bring up difficulties with eating, sleep, pain, energy, work or caring for someone. The team can arrange symptom support while cancer treatment continues.

Skin, body changes and daily care

Use the detailed body map to locate symptoms, not to diagnose them.

The body map

The organ anchors show possible locations and warning symptoms. They do not show how common each effect is, and a symptom may have another cause. New symptoms should be assessed in their clinical context.

Food, fluids and activity

Choose food, fluids and movement that you can manage, within any personal restrictions. Ask for help early if you are losing weight, drinking less or finding everyday activity harder.

Keep checking the plan

A new medicine, fever, hospital admission, procedure or radiotherapy may change treatment timing. Tell the oncology team and take the wallet card or medicine list to other appointments.

Fertility, pregnancy and breastfeeding

Discuss these issues before the first dose whenever possible.

Fertility preservation

Ask whether treatment or the cancer may affect fertility and whether fertility preservation is appropriate before treatment. A change in periods or sexual function does not reliably establish infertility.

Written dates and methods

Record the agreed method, last treatment date and how long precautions continue. Different medicines have different intervals, so ask for clarification after a switch or combination change.

Sexual health

You can request a private discussion about intimacy, sexual symptoms or relationships. Ask for help if the agreed pregnancy precautions are difficult to follow.

Questions worth asking

Which tests are needed?

Confirm BRAF V600, blood count, kidney/liver tests, medicine interactions and skin assessment. Current Zelboraf product information calls for a baseline ECG and electrolytes including magnesium for all patients; this supplements the NCCP risk-based ECG row. Staging scans establish the sites of melanoma. Blood count, kidney and liver tests before each cycle. NCCP specifies ECG every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Its skin-review row states week eight then every 12 weeks; current Zelboraf product information instead requires monthly prescriber skin examinations. The team should supply a reconciled written skin-check plan using the current product information. Current Zelboraf product information calls for monthly skin checks during treatment and for up to six months after stopping, or until another anticancer therapy starts. Head-and-neck checks every three months and chest CT every six months during treatment screen for other cancers; these are distinct from the individual melanoma-response scan plan. Anal/pelvic checks are considered at baseline/end and when indicated. Record the team's coordinated surveillance plan rather than duplicating scans.

Can I use this without a phone?

Yes. The complete handbook, three-page summary and cut-out wallet card are printable. Ask the team to enter actual contact numbers and provide an updated paper plan when anything changes.

Try explaining the plan back: Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

The treatment this guide covers This is vemurafenib monotherapy. It is not the cobimetinib combination and has no cobimetinib seven-day break. The team must plan any radiotherapy because BRAF treatment can increase radiation-related reactions.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Vemurafenib alone — advanced melanoma · support plan

Active medicines: Vemurafenib

Emetogenic risk: Minimal per NCCP 00102 v6; do not replace with the combination classification

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- This is a minimal-emetic-risk oral regimen in the NCCP plan. Anti-sickness medicine is chosen to match symptoms and interactions; do not add over-the-counter medicines without checking. Agree a hydration and diarrhoea plan, and report inability to drink or persistent symptoms.
- Blood count, kidney and liver tests before each cycle. NCCP specifies electrocardiogram (ECG) every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Its skin-review row states week eight then every 12 weeks; current Zelboraf product information instead requires monthly prescriber skin examinations. The team should supply a reconciled written skin-check plan using the current product information.
- Contact the oncology team promptly for fainting or palpitations, eye pain or blurred vision, a blistering or rapidly spreading rash, severe sunburn, persistent vomiting or diarrhoea, reduced urine or yellow skin or eyes. Report new or changing skin growths even when the melanoma is responding.
- Use effective contraception during treatment and for at least six months after the final dose. Vemurafenib may reduce the effectiveness of hormonal contraception, so ask for an appropriate alternative or additional method. Discuss fertility and breastfeeding before treatment; tell the team immediately if pregnancy is possible.
- This is vemurafenib monotherapy. It is not the cobimetinib combination and has no cobimetinib seven-day break. The team must plan any radiotherapy because BRAF treatment can increase radiation-related reactions.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Head / fatigue

Vemurafenib

Headache and tiredness can occur. New severe headache, confusion, collapse or neurological changes need urgent assessment rather than attribution to tablets.

3 Heart rhythm

Vemurafenib

The electrical rhythm can change; ECG and blood chemistry checks help detect risk. Palpitations or fainting need prompt assessment.

5 Liver

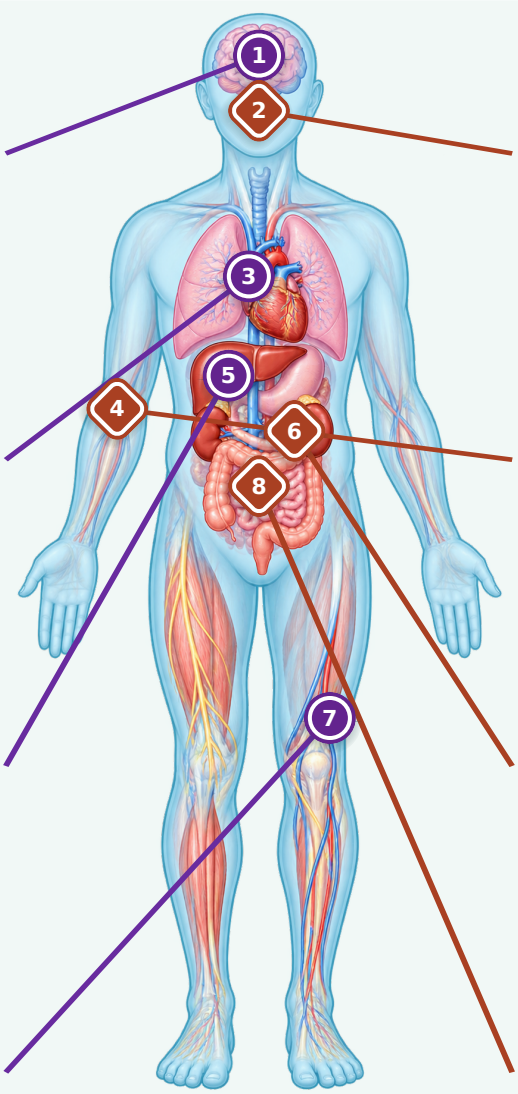
Vemurafenib

Blood tests check for liver injury. Yellow skin or eyes, dark urine or severe upper-abdominal discomfort need prompt review.

7 Joints / muscles

Vemurafenib

Joint or muscle pain can affect daily activities. Tell the team if pain limits walking or self-care, or occurs with marked weakness.



2 Eyes

Vemurafenib

Eye pain, redness, light sensitivity or blurred vision can signal inflammation. Report promptly for eye assessment.

4 Skin / sun

Vemurafenib

Photosensitivity, rash and new skin cancers can occur. Protect skin and report new growths; blistering or peeling is urgent.

6 Kidneys / fluids

Vemurafenib

Kidney injury or dehydration can affect dosing. Report reduced urine or inability to keep fluids down.

8 Stomach / bowel

Vemurafenib

Nausea, diarrhoea and reduced appetite can cause dehydration. Keep the agreed symptom plan and report persistent or severe symptoms.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team promptly for fainting or palpitations, eye pain or blurred vision, a blistering or rapidly spreading rash, severe sunburn, persistent vomiting or diarrhoea, reduced urine or yellow skin or eyes. Report new or changing skin growths even when the melanoma is responding.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00102 v6 supportive-care section; current product information for medicine-specific precautions**
   https://healthservice.hse.ie/documents/6723/102_v6_Vemurafenib_Monotherapy.pdf
2. **EMA Zelboraf product information ↗**
   https://www.ema.europa.eu/en/documents/product-information/zelboraf-epar-product-information_en.pdf
3. **Chapman et al. BRIM-3, NEJM 2011**
   https://www.nejm.org/doi/full/10.1056/NEJMoa1103782
4. **NCI: fatigue during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/fatigue
5. **NCI: melanoma — patient information**
   https://www.cancer.gov/types/skin/patient/melanoma-treatment-pdq
6. **NCI: melanoma treatment — clinical evidence**
   https://www.cancer.gov/types/skin/hp/melanoma-treatment-pdq
7. **NCI: eating and nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
8. **NCI: palliative care during cancer treatment**
   https://www.cancer.gov/about-cancer/advanced-cancer/care-choices/palliative-care-fact-sheet